BASELINE → TREATMENT → FOLLOW-UP

When your checks happen

Cycle numbers are examples for uninterrupted three-weekly treatment. Schedule checks by elapsed time and clinical need. Moving to maintenance does not restart the cardiac-monitoring clock. The clinical team books and reviews results; this map does not track completed tests.

Check	Before this phase	Before every 21-day cycle	Week 12 · around C5	Weeks 24 / 36 · around C9 / C13	Later / treatment change
Heart pumping test · echo / multigated acquisition (heart pumping) scan (MUGA)	Baseline cardiac assessment and left ventricular ejection fraction (heart pumping measurement) (LVEF) before T-DM1	Review symptoms; earlier echo if clinically indicated	Echo / MUGA at 12 weeks, around C5	Repeat around C9 and C13 if uninterrupted	Review at completion; agree follow-up based on cardiac risk and findings.
Blood tests	Baseline full blood count (FBC), renal and liver profile	FBC, renal and liver tests before every cycle	Continue pre-cycle blood tests	Continue pre-cycle blood tests	Follow abnormalities and the next treatment plan
Cancer imaging / markers	Surgery and pathology establish the indication	Review new symptoms at every visit	No routine computed tomography (CT) series if well in early breast follow-up	Agree breast surveillance and any symptom-led tests	Annual mammography of remaining breast tissue; additional imaging if indicated
electrocardiogram (ECG) / bone health	ECG when indicated by cardiac assessment; dual-energy X-ray absorptiometry (bone density) (DEXA) only if endocrine / fracture risk indicates	No automatic every-cycle ECG or DEXA	Risk-based clinical plan	Risk-based clinical plan	Keep any separately prescribed endocrine and bone-health follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00659 v2 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up

THE WHOLE TREATMENT JOURNEY

Adjuvant trastuzumab emtansine (T-DM1): the whole pathway

Adjuvant · residual invasive HER2-positive early breast cancer

T-DM1 by intravenous (IV) infusion every 21 days for 14 postoperative cycles, unless recurrence or unacceptable toxicity requires a change.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before surgery	Surgery / pathology	T-DM1 · 14 cycles	Completion	Follow-up
Cancer medicines NCCP 00659	Prior taxane + HER2 treatment	Confirm residual invasive disease	T-DM1 every 21 days	Review final dose / unresolved effects	Endocrine therapy continues if indicated
Surgery EARLY	Neoadjuvant treatment precedes operation	Lumpectomy / mastectomy and node surgery	Wound and arm rehabilitation	Review local symptoms	Long-term local surveillance
Radiotherapy EARLY	multidisciplinary team (MDT) assesses likely need	Pathology determines indication	radiotherapy (RT) may overlap T-DM1 under MDT plan	Report cough / breathlessness	Assess possible late lung / cardiac effects
Blood tests NCCP 00659	Prior-treatment checks	Baseline FBC, liver and renal tests	FBC including platelets, renal and liver tests before every cycle	Review abnormal results	As required by continuing care
Heart checks NCCP 00659 · ESC	Baseline / earlier HER2 monitoring	Review LVEF before switching	Every 12 weeks: around cycles 5, 9 and 13 if uninterrupted; earlier if needed	Completion assessment	Risk-based surveillance
Cancer imaging EARLY · BONE	Response imaging for surgery	Pathology establishes this indication	No routine CT series if well	Agree mammogram schedule	Annual mammography of remaining breast tissue

What each check is for

1 Blood tests and treatment review

Prior-treatment checks → Baseline FBC, liver and renal tests → Before every cycle: platelets, liver and renal tests → Review abnormal results → As required by continuing care

Results may change whether a dose is given or held.

Ask / act: Confirm the booking and who reviews the result before treatment.

Sources: NCCP 00659

2 Heart tests

Baseline / earlier HER2 monitoring → Review LVEF before switching → Every 12 weeks; sooner if abnormal / symptoms → Completion assessment → Risk-based surveillance

ECG measures rhythm; echo measures pumping function. They are different checks.

Ask / act: Keep the planned appointments and report symptoms between them.

Sources: NCCP 00659 · ESC

3 Imaging and markers

Response imaging for surgery → Pathology establishes this indication → No routine CT series if well → Agree mammogram schedule → Annual mammography of remaining breast tissue

The purpose and frequency depend on early versus advanced disease.

Ask / act: Ask which test is next and what decision its result will inform.

Sources: EARLY · BONE

4 Bone health with endocrine treatment

At endocrine / ovarian-suppression planning; repeat by risk

Low oestrogen can affect bone strength.

Ask / act: Ask whether DEXA, fracture-risk assessment or bone protection is appropriate.

Sources: BONE · NOGG

5 Who arranges and reviews each check?

At initiation and every handover

A recommended test can be missed if responsibility is unclear.

Ask / act: Ask the team to document the booking owner, result reviewer, due window and escalation route.

Sources: NCCP 00659

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.